1. PATIENT PROFILE & COMPOSITE ABILITY INDICES

Candidate ID: PT-NEURO-113	**Chronological Age:** Adult Norms (18-45)	**Normative Reference:** CHC Standardization
FSIQ Standard Score: **113** (78.8th Percentile)	**General Ability Index (GAI):** **120 - 121** (91st %)	**95% Confidence Interval:** 108 — 118

Factor Domain	CHC Symbol	Index Score	Percentile	Classification	Clinical Interpretation
Fluid Reasoning	Gf	123	93.7th	Superior	Rapid inductive reasoning, novel pattern extraction
Working Memory	Gwm	114	82.5th	High Average	Strong spatial retention, stable reverse Corsi span
Quantitative Reasoning	Gq	109	72.6th	High Average	Accurate numerical extrapolation & series induction
Processing Speed	Gs	78	7.1th	Significant Split	Neuro-motor latency vulnerability; triggers GAI diver

CLINICAL DISCREPANCY ANALYSIS (GAI ELEVATION): A statistically significant **45-point discrepancy** is verified between Superior Fluid Reasoning (Gf = 123) and Motor Processing Speed (Gs = 78). Under standard APA/WISC psychometric guidelines, FSIQ (113) underestimates the candidate's core abstract intellectual capacity. The **General Ability Index (GAI = 120-121)** is certified as the clinically unitary representation of higher-order cognitive ability.

2. 40-ITEM SOLVED PROBLEM TAXONOMY & LATENCY TELEMETRY

Subtest Domain	Attempted	Accuracy	Mean Latency	Primary Error Mode	Resolution Protocol
Fluid Matrix (Gf)	8 of 8	100.0%	1.42s / item	None (Perfect Inductive)	Maintained high abstraction focus
Spatial Memory (Gwm)	8 of 8	87.5%	0.85s / flash	Reverse Span Level 6	Chunking into 3-block vectors
Quantitative (Gq)	6 of 6	100.0%	2.10s / item	None (Arithmetic Extrap.)	Pattern-difference verification
Processing Speed (Gs)	18 bursts	83.3%	0.62s / probe	Motor click latency delay	Hardware hotkey binding (Y/N)

3. CLINICAL PSYCHIATRIC SCREENER RESULTS (DSM-5 ALIGNED)

Multi-dimensional behavioral, neurodevelopmental, and clinical screener indices:

Clinical Screener	Inventory Tool	Raw Score	Cutoff	Clinical Severity	Diagnostic Impression
Adult Attention (ADHD)	ASRS v1.1 (WHO)	14 / 24	≥ 14	Elevated Inattention	Sub-clinical executive focus bottleneck; motor spee
Obsessive-Compulsive (OCD)	OCI-R Standard	11 / 72	≥ 21	Minimal Risk	High mental orderliness without compulsive ritual in
Schizotypy Spectrum	SPQ-B Diagnostic	7 / 22	≥ 12	Sub-clinical	Elevated divergent ideation; intact reality testing
Autism Spectrum Quotient	AQ-10 Screener	4 / 10	≥ 6	Typical / Negative	High systemizing focus with intact socio-pragmatic
Generalized Anxiety (GAD)	GAD-7 Scale	6 / 21	≥ 10	Mild Tension	Anticipatory cognitive strain under motor time press
Depressive Symptomatology	PHQ-9 Screener	3 / 27	≥ 10	Minimal / None	Robust affective stability and euthymic baseline
Executive Function Profile	Real-World EF-14	9 / 28	≥ 12	Controlled	Occasional initiation friction; compensated by high
Trauma & Acute Stress	PCL-5 DSM-5	6 / 32	≥ 14	Low / Non-clinical	Absence of intrusive flashback or hyperarousal syn

4. ACTIONABLE ENHANCEMENT ROUTINES FOR PATIENT & PRACTITIONER

PATIENT DAILY COGNITIVE PROTOCOL (OPTIMIZED FOR IQ 113 & Gf/Gs DISCREPANCY):

- **08:00 - Morning Dopamine & Focus Priming:** 200mg L-Theanine + 100mg Caffeine or cold splash. Zero phone/notifications for 45 mins to protect prefrontal cortex working memory buffer.
- **09:00 - High-Abstraction Deep Work Block:** 90-minute uninterrupted session utilizing superior Gf (123). Leverage keyboard hotkeys, large monitors, and zero-click workflows to eliminate motor latency frustration.
- **14:00 - Dual N-Back Neuroplasticity Training:** 15 minutes of Dual N-Back (N=2 to N=3) targeting working memory updating and auditory-spatial integration.
- **21:30 - Sensory Decompression & Sleep Hygiene:** Blue light block, magnesium glycinate (300mg), zero screen stimuli 45 mins prior to sleep to reset neurochemical speed buffers.

PRACTITIONER CLINICAL ROADMAP & ACCOMMODATIONS:

- **Diagnostic Accommodation:** Recommend 1.5x time allowances on timed motor/processing examinations. Never interpret raw processing speed as representative of intellect.
- **Executive Scaffolding:** Deploy automated checklist templates and voice-to-text dictation tools to bypass motor transcription friction.
- **Split-Index Certification:** Emphasize GAI (120-121) in all educational, occupational, and clinical evaluations rather than FSIQ.

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